

Chest Injury - Female (Child)

Algorithm 1248 | Child (0-17 years) | Gender at birth: Female

Definition

- Chest injury (broken or bruised ribs) assessment decomposed from NHS.UK's published when-to-get-help guidance.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What is the person's exact age and sex, are they pregnant, taking an anticoagulant, or medically fragile, and exactly how did the injury happen? For a child or vulnerable person, follow the approved safeguarding pathway if the account or injury pattern is inconsistent.
2. [DURATION] When did it happen?
3. [YES_NO] Any shortness of breath?
4. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Redirect → Motor Vehicle Accident

Was the chest injury part of a motor vehicle crash or a multi-system crash presentation?

A motor vehicle crash with possible injuries in more than one body area belongs in the Motor Vehicle Accident pathway.

1. Disposition level 100

Was this caused by a serious accident such as a car accident, is shortness of breath getting worse, is chest pain getting worse, is there pain in the tummy or shoulder, or is the person coughing up blood?

NHS.UK broken/bruised ribs guidance lists these as emergency-assessment criteria - could mean a rib has damaged the lung, liver, or spleen.

1. Disposition level 70

For a child or vulnerable person, is the explanation absent, unsuitable, inconsistent with the injury or developmental ability, is there delay in seeking help, might the caregiver be unsafe, or can the patient not be offered a safe opportunity to speak privately?

NICE child-maltreatment guidance identifies unsuitable or inconsistent explanations and caregiver obstruction of private discussion as safeguarding alerts. Exact Qatar reporting, destination, confidentiality, and disconnect actions remain GOVERNANCE_REQUIRED.

1. Disposition level 70

Has the pain not improved within a few weeks, is there yellow or green mucus when coughing, or a very high temperature or feeling hot, cold, or shivery?

NHS.UK guidance recommends urgent clinical review for these features - may need stronger pain relief or antibiotics for a chest infection.

1. Disposition level 15

Is this mild pain or bruising with none of the features above?

NHS.UK guidance: broken/bruised ribs usually get better on their own within 2-6 weeks with self-care.

Advice

Qatar emergency chest-injury precautions

- Call Qatar emergency services on 999 and do not drive. Keep the person still in the position that makes breathing easiest; do not tightly wrap or bind the chest and do not give food, drink, or unprescribed medicine. Control an open wound with a clean dressing without sealing or pressing on an embedded object. If unresponsive and not breathing normally, start age-appropriate CPR as directed by 999. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: worsening breathing difficulty; increasing pain; coughing blood; faintness, confusion, or abnormal colour.

Child chest-injury safeguarding response

- Arrange urgent in-person clinical and safeguarding assessment through the Qatar governance-approved pathway. Offer the child a safe opportunity to speak privately when clinically and developmentally appropriate; do not rely on or disclose the plan to a caregiver who may be unsafe. Do not confront a suspected abuser. If the child is in immediate danger, has life-threatening injury, or contact is lost before safety is established, use 999 and the approved emergency handover or welfare-check procedure. Exact reporting, consent, confidentiality, documentation, destination, and disconnect actions remain GOVERNANCE_REQUIRED.
- Call Back If: immediate danger or unsafe caregiver; new breathing difficulty, collapse, or severe pain; caller disconnects before safety is established.

Urgent chest injury review

- Arrange same-day medical review through the approved Qatar pathway for possible chest infection or additional pain relief.

- Call Back If: breathing worsens; fever develops.

Governed care for a minor chest injury

- Only when injury and safeguarding concerns are absent, do not bind the chest. Rest from provoking activity but change position and breathe normally; support the sore area with a pillow when coughing. A wrapped cool pack may be used briefly without direct skin contact. Medicine choice and dose require age, weight, pregnancy, allergy, kidney/liver disease, ulcer/bleeding risk, anticoagulant, and current-medicine checks under the approved Qatar pathway.
- Call Back If: breathing difficulty develops; pain worsens; fever, coloured sputum, faintness, or coughing blood; safeguarding concern develops.

First aid

- Call Qatar emergency services on 999 and do not drive. Keep the person still in the position that makes breathing easiest; do not tightly wrap or bind the chest and do not give food, drink, or unprescribed medicine. Control an open wound with a clean dressing without sealing or pressing on an embedded object. If unresponsive and not breathing normally, start age-appropriate CPR as directed by 999. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Broken or bruised ribs", <https://www.nhs.uk/conditions/broken-or-bruised-ribs/> (page last reviewed 10 January 2024)
- NICE, "Child maltreatment: when to suspect maltreatment in under 18s", <https://www.nice.org.uk/guidance/cg89/chapter/recommendations>
- NICE, "Child abuse and neglect", <https://www.nice.org.uk/guidance/ng76/chapter/recommendations>

Provenance

Shaped to mirror the licensed After-Hours Telehealth Triage Guidelines database structure. This is IST open-source guideline content derived from cited public sources; it is not licensed STCC content. Question.DispositionLevel=null marks a See More Appropriate Guideline redirect. Demographic variants contain only source-supported differences; identical thresholds are documented rather than fabricated. Local destinations and TelemedicineEligible values require IST clinical governance approval. UAT DATA ONLY - NOT FOR REAL-PATIENT CARE OR PRODUCTION USE.

Manifest hash: 52d590ab9946967eb7b497effcfe81088f1e838caf8234fc385871861a679e32 | Status: BLOCKED_BY_RESEARCH_GAP